

pdac-recurrent-cldn18-2-kras-g12r-tp53 -y220c-p25m

Standard of care options — approved and guideline-endorsed strategies

Generated 2026-08-12

Standard of care options

The treatment strategies that are standard for this patient's situation, meaning a regulator approved them for a population that includes this patient or a major academic or clinical-society guideline carries them. This report runs alongside the targetable-feature ranking rather than in place of it, and it does not narrow what that ranking surfaces.

Assessed: 12 standard options. **Consider now:** 6. **Behind an open gate:** 2. **Already received:** 1.

Twelve standard options were assessed for this recurrence. Six are open to consider now, two sit behind workup gates, two belong to a later decision point, one has already been given, and one was set aside. The honest headline is that the standard of care here is thin. NCCN's own pathway marks a clinical trial as preferred for a recurrence inside 6 months of adjuvant completion, and this one came at about 4 months. The default chemotherapy is a class switch to a gemcitabine-based regimen, in US practice usually gemcitabine with nab-paclitaxel, and the right benchmark for it is the post-FOLFIRINOX AGEO cohort: response rate 17.5%, median survival 8.8 months (95% CI 6.2-9.7). ESMO's April 2025 Express Update grades gemcitabine plus conventional paclitaxel I, B in this position and gemcitabine/nab-paclitaxel III, C; no grading changes how modest all three are. NCCN was verified at v2.2026; v3.2026 has posted and could not be read.

Already spent: 10 cycles of FOLFIRINOX across the neoadjuvant and adjuvant settings, which is why neither FOLFIRINOX re-challenge nor NALIRIFOX, an approved first-line regimen built from the same three drug classes, appears as a live option.

Gated: radiotherapy waits on the staging call that would make this an isolated local recurrence; olaparib waits on the germline panel that has never been drawn; and every taxane-containing row waits on a CTCAE grade for the residual oxaliplatin neuropathy.

This report adds the standard options and does not narrow the experimental options ranked elsewhere on this page. The workup at the top of both tables is the same step, and the one hard conflict between the tracks is timing: starting chemotherapy spends the six-week RASolute 303 consent window.

Options to consider now

Standard options this patient is eligible for, with an endorsement that covers their situation, and no unresolved gate in front of them.

Biopsy confirmation of the recurrence, restaging, tumor NGS, and dedicated germline testing

Priority: essential **Modality:** Other **Line:** Not line-indexed **Intent:** disease-control intent

What makes it standard.

- NCCN Guideline workup mandate (universal germline testing since 2019; tumor NGS for treatment candidates; suspected recurrence confirmed by imaging and/or biopsy) (covers this patient · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 has since posted and could not be read))
 - covers: All patients with pancreatic adenocarcinoma: dedicated germline testing; tumor/somatic NGS for

every patient who is a candidate for anti-cancer therapy; suspected recurrence after resection confirmed by imaging and/or biopsy before treatment (March 2026 update).

- ASCO Guideline recommendation (germline and somatic testing early for all treatment-eligible patients) (covers this patient · ASCO Guideline Update 2020)
 - covers: Metastatic pancreatic cancer, all treatment-eligible patients: germline testing plus somatic testing for MSI/dMMR, BRCA, and NTRK alterations, used to select patients for pembrolizumab, olaparib, larotrectinib, or entrectinib.

Fit to this patient. Eligible Nothing in the profile blocks the workup: ECOG 0, and the germline panel and neuropathy grading are blood-and-exam items that need no tissue. The one physical constraint is procedural, the likely vascular invasion around the mass, which shapes the biopsy route rather than whether confirmation is owed.

Everything known about this tumor comes from the December 2025 resection specimen, and the guideline asks for three things before treatment: confirmation of the suspected recurrence by imaging and/or biopsy, tumor NGS for a treatment candidate, and the dedicated germline panel NCCN has mandated in every PDAC patient since 2019, which incidental tumor-NGS reporting does not satisfy. The same workup produces the staging call that decides which standard and which experimental options are open. This is guideline work, not extra caution.

Key evidence.

- Groot 2019 (Johns Hopkins recurrence cohort) · retrospective single-center cohort · n=957 · Recurrence within 12 months of resection carried 20% 1-year post-recurrence survival vs 45% for later recurrence (P<0.001)

Toxicities that would change the decision. Procedural risk of EUS core sampling (bleeding, post-procedure pancreatitis), raised by the likely vascular invasion around the mass

Alongside the targeted options. Comes before the targeted options. This is the same step the experimental ranking carries at rank 1. The staging call opens or closes the daraxonrasib expanded-access protocol, RASolute 303, and the ASP2138 monotherapy arms; the germline draw closes the olaparib gate on both tables; and the neuropathy grade decides taxane exposure on both tracks.

Last verified. 2026-08-12

References. guideline:NCCN Pancreatic Adenocarcinoma v2.2026; pmid:32755482; pmid:30589608; pmid:31082915

Gemcitabine + nab-paclitaxel

Priority: high **Modality:** Systemic chemotherapy **Line:** Second line **Intent:** life-prolonging intent

What makes it standard.

- NCCN Recurrence pathway: alternative systemic regimen for recurrence within 6 months of completing adjuvant therapy (gemcitabine-based after fluoropyrimidine-based therapy); clinical trial preferred in this setting (covers this patient · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Recurrence after resection and adjuvant therapy: recurrence within 6 months of completing adjuvant therapy is treated with an alternative regimen not previously used; after fluoropyrimidine-based therapy that means a gemcitabine-based regimen.
- ESMO III, C as second-line after FOLFIRINOX (Express Update; not EMA- or FDA-approved as second-line therapy). ESMO-MCBS 3 applies to the first-line indication (vs gemcitabine) (partial population match · ESMO

Express Update, April 2025)

- covers: Metastatic pancreatic cancer after first-line FOLFIRINOX: gemcitabine-nab-paclitaxel is an option; the graded I, B choice in this position is gemcitabine plus conventional paclitaxel for ECOG PS 0-2.
- FDA On-label approval (nab-paclitaxel in combination with gemcitabine) (partial population match · FDA approval 2013-09)
 - covers: First-line treatment of patients with metastatic adenocarcinoma of the pancreas, in combination with gemcitabine.

Fit to this patient. Conditionally eligible No component repeats what the tumor progressed through, which is the point of the class switch. The gates are in the record: the residual oxaliplatin neuropathy after 10 FOLFIRINOX cycles has no CTCAE grade and nab-paclitaxel adds taxane neurotoxicity on top of it, no organ-function labs exist since the resection, and the recurrence is not yet biopsy-confirmed.

- blocker: Residual oxaliplatin neuropathy after 10 FOLFIRINOX cycles is ungraded, and nab-paclitaxel stacks taxane neurotoxicity on the same axis
- blocker: No post-resection renal, hepatic, or marrow labs on file
- blocker: The recurrence is radiographic only; the guideline asks for confirmation before treatment

This is the standard class switch for disease that has progressed through FOLFIRINOX, and the NCCN recurrence pathway routes a recurrence inside 6 months of adjuvant completion, which is this case at about 4 months, to exactly this kind of gemcitabine-based regimen. Expectations should come from the post-FOLFIRINOX data rather than the first-line label: ORR 17.5% and median OS 8.8 months (95% CI 6.2-9.7) in the AGEO cohort. The open items before starting are the neuropathy grade, baseline organ-function labs, and biopsy confirmation of the recurrence.

Key evidence.

- AGEO cohort (Portal 2015) · prospective multicenter cohort, 12 French centers · n=57 · Median OS 8.8 months (95% CI 6.2-9.7); PFS 5.1 months (95% CI 3.2-6.2); ORR 17.5%; grade 3-4 toxicity 40%, neurotoxicity 12.5%
- MPACT (Von Hoff 2013) · randomized phase 3, first-line metastatic · n=861 · Median OS 8.5 vs 6.7 months, HR 0.72 (95% CI 0.62-0.83), P<0.001; the approval basis, in a different line than this patient's

Toxicities that would change the decision. Grade 3-4 neurotoxicity 12.5% in the AGEO post-FOLFIRINOX cohort, landing on top of ungraded residual oxaliplatin neuropathy; Grade 3-4 toxicity 40% overall in AGEO (neutropenia 12.5%, asthenia 9%, thrombocytopenia 6.5%) with marrow reserve undocumented after 10 chemotherapy cycles

Alongside the targeted options. May foreclose a targeted option. RASolute 303 requires consent within 6 weeks of the metastatic diagnosis, so starting this regimen first spends that window, and it changes the line count several CLDN18.2 protocols screen on. In the other direction, its AGEO benchmark (median OS 8.8 months) is the floor the experimental rows are measured against, and it remains available after a trial.

Last verified. 2026-08-12

References. guideline:NCCN Pancreatic Adenocarcinoma v2.2026; pmid:40287191; pmid:26372701; pmid:24131140; fda:nab-paclitaxel-metastatic-pancreatic-adenocarcinoma-2013-09-06

Early palliative care and a goals-of-care discussion

Priority: high **Modality:** Supportive care **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- ASCO Guideline recommendation (2024 update) (covers this patient · ASCO Guideline Update 2024)
 - covers: Patients with advanced cancer: dedicated palliative care services early in the disease course, concurrent with active treatment.
- NCCN Supportive-care principles (pancreatic guideline) (covers this patient · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Advanced or recurrent pancreatic cancer: symptom management and palliative care integrated with disease-directed treatment.

Fit to this patient. Eligible No profile fact bears on eligibility; the recommendation applies to every patient in this disease state, at any performance status.

A recurrence 4 months after completing adjuvant therapy for pancreatic cancer is the situation the early palliative care evidence was built for, and ASCO's recommendation is concurrent with treatment, not instead of it. The preferences on file weight efficacy at 0.7 as an assumed default, and the intake itself flags that goals of care are undocumented; a goals-of-care conversation is how that number stops being an assumption. This row competes with nothing on either table.

Alongside the targeted options. Can run alongside the targeted options. Explicitly concurrent: choosing aggressive treatment, a trial, or neither does not change the recommendation. Documenting goals of care would also firm up the efficacy/toxicity weighting the rest of the case currently carries as an assumed default.

Last verified. 2026-08-12

References. pmid:38748941; guideline:NCCN Pancreatic Adenocarcinoma v2.2026

Pancreatic enzyme replacement therapy and nutritional support

Priority: high **Modality:** Supportive care **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- NCCN Supportive-care recommendation (covers this patient · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Pancreatic exocrine insufficiency after pancreatectomy or from pancreatic cancer: pancreatic enzyme replacement with nutritional assessment and support.
- other_society AGA Clinical Practice Update, Best Practice Advice (2023) (covers this patient · AGA 2023)
 - covers: Exocrine pancreatic insufficiency, with pancreatic resection among the high-risk settings: diagnose, treat with pancreatic enzyme replacement, and monitor nutrition including fat-soluble vitamins and bone health.
- FDA On-label approval (pancrelipase products) (covers this patient · FDA approvals 2009 onward (pancrelipase class))
 - covers: Treatment of exocrine pancreatic insufficiency, including insufficiency due to pancreatectomy.

Fit to this patient. Likely eligible The record describes the resection as a Whipple (per the user; the operative report was not supplied), which makes exocrine insufficiency probable, but no fecal elastase, weight trend, or symptom record is on file to say whether replacement is already in place or adequate.

After a pancreatic resection, and with a new mass in the remnant, exocrine insufficiency is the default assumption

until tested, and untreated insufficiency quietly erodes the weight, albumin, and performance status every treatment plan on this page depends on. Enzyme replacement is inexpensive, on-label, and carried by both NCCN and the AGA. Glycemic surveillance for post-pancreatectomy diabetes belongs in the same visit.

Toxicities that would change the decision. Minimal at standard dosing; the practical failure mode is underdosing, not toxicity

Alongside the targeted options. Can run alongside the targeted options. Runs alongside any systemic, radiotherapy, or trial route and conflicts with none of them. Nutritional state feeds the screening labs and performance-status criteria every protocol on the experimental table applies.

Last verified. 2026-08-12

References. guideline:NCCN Pancreatic Adenocarcinoma v2.2026; pmid:37737818; fda:pancrelipase-exocrine-pancreatic-insufficiency

Gemcitabine + paclitaxel (GEMPAX regimen)

Priority: medium **Modality:** Systemic chemotherapy **Line:** Second line **Intent:** life-prolonging intent

What makes it standard.

- ESMO I, B (Express Update) (partial population match · ESMO Express Update, April 2025)
 - covers: After FOLFIRINOX treatment in metastatic pancreatic cancer, gemcitabine plus paclitaxel can be offered to patients with an ECOG PS of 0-2 and a favourable comorbidity profile.

Fit to this patient. Conditionally eligible ECOG 0 fits the ESMO wording, but the recommendation was written for metastatic disease after first-line FOLFIRINOX, and this patient's exposure was perioperative with the recurrence still unstaged. The taxane gate is the same as for nab-paclitaxel: the residual oxaliplatin neuropathy has no grade on file.

- blocker: Ungraded residual oxaliplatin neuropathy; grade ≥ 3 neuropathy reached 12.3% of the GEMPAX doublet arm
- blocker: No post-resection organ-function labs
- blocker: Recurrence not yet biopsy-confirmed or staged

The only randomized grading ESMO gives in this exact position belongs to this doublet: I, B after FOLFIRINOX for ECOG PS 0-2, resting on the GEMPAX trial. The honest reading of GEMPAX is mixed, because the OS primary endpoint was not met (6.4 vs 5.9 months, HR 0.87) while PFS and response improved. NCCN does not carry this specific doublet, and in US practice the gemcitabine-based switch usually means gemcitabine with nab-paclitaxel.

Key evidence.

- GEMPAX / PRODIGE 65 (De La Fouchardiere 2024) · randomized phase 3, open-label, 2:1 · n=211 · Median OS 6.4 vs 5.9 months, HR 0.87 (95% CI 0.63-1.20), P=0.41, primary endpoint not met; PFS 3.1 vs 2.0 months, HR 0.64 (95% CI 0.47-0.89); ORR 17.1% vs 4.2%; grade ≥ 3 TRAEs 58.0% vs 27.1%

Toxicities that would change the decision. Grade ≥ 3 neuropathy 12.3% in the doublet arm, the wrong toxicity to add to ungraded oxaliplatin neuropathy; Grade ≥ 3 treatment-related AEs 58.0% vs 27.1% for gemcitabine alone

Alongside the targeted options. Competes for the same line of therapy. Occupies the same second-line slot as the ranked experimental options, and like any chemotherapy start it spends the RASolute 303 six-week consent window.

Last verified. 2026-08-12

References. pmid:40287191; pmid:38232341

Gemcitabine monotherapy

Priority: medium **Modality:** Systemic chemotherapy **Line:** Second line **Intent:** life-prolonging intent

What makes it standard.

- FDA On-label approval (covers this patient · FDA approval 1996-05)
 - covers: First-line treatment of patients with locally advanced (nonresectable stage II or III) or metastatic (stage IV) adenocarcinoma of the pancreas; indicated for patients previously treated with fluorouracil.
- ESMO III, C (Express Update) (partial population match · ESMO Express Update, April 2025)
 - covers: Metastatic pancreatic cancer after FOLFIRINOX: gemcitabine alone is an option.
- NCCN Recurrence pathway: gemcitabine-based regimen after prior fluoropyrimidine-based therapy (covers this patient · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Recurrence within 6 months of completing adjuvant fluoropyrimidine-based therapy: an alternative, gemcitabine-based regimen.

Fit to this patient. Likely eligible The one systemic option on this page that adds no neurotoxic component, so the ungraded oxaliplatin neuropathy does not gate it. What is missing is baseline marrow, renal, and hepatic labs, unmeasured since the resection.

Single-agent gemcitabine is the fallback inside the same guideline pathway, and its FDA label covers this patient as written, since it is indicated for pancreatic adenocarcinoma previously treated with fluorouracil. It is the least active choice in this position (median OS 5.9 months as the GEMPAX control arm) and the only one that leaves the neuropathy axis alone. In practice the neuropathy grade decides between this row and the taxane doublets.

Key evidence.

- Burris 1997 · randomized phase 3 vs bolus 5-FU · n=126 · Clinical benefit 23.8% vs 4.8%; median OS 5.65 vs 4.41 months (P=0.0025)
- GEMPAX control arm (2024) · randomized phase 3 control arm, second line after FOLFIRINOX · n=71 · Median OS 5.9 months (95% CI 4.6-6.9); PFS 2.0 months; ORR 4.2%; the contemporary estimate of what gemcitabine alone does in exactly this position

Toxicities that would change the decision. Myelosuppression and fatigue, with marrow reserve undocumented after 10 chemotherapy cycles; Spares the neuropathy axis entirely, which no taxane-containing row does

Alongside the targeted options. Competes for the same line of therapy. Same second-line slot as the ranked options; starting it spends the RASolute 303 consent window like any other chemotherapy start.

Last verified. 2026-08-12

References. fda:gemcitabine-pancreatic-adenocarcinoma-1996; pmid:40287191; pmid:9196156; pmid:38232341

Options at the next decision point

Standard for a line this patient has not reached yet, or for a disease state that has not arrived. Listed now so the sequence is visible in advance.

Liposomal irinotecan + 5-FU/leucovorin (NAPOLI-1 regimen)

Priority: medium **Modality:** Systemic chemotherapy **Line:** Third line or later **Intent:** life-prolonging intent

What makes it standard.

- FDA On-label approval (2015) (different population · FDA approval 2015-10)
 - covers: Metastatic adenocarcinoma of the pancreas after disease progression following gemcitabine-based therapy, in combination with fluorouracil and leucovorin.
- NCCN Subsequent-therapy option after gemcitabine-based therapy (different population · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Advanced pancreatic adenocarcinoma previously treated with gemcitabine-based therapy.

Fit to this patient. Likely ineligible Not an option today: the label and the guideline slot both sit after gemcitabine-based therapy, which this patient has not yet received. It becomes the on-label next step if a gemcitabine-based second line is used and fails.

- blocker: Label requires prior progression on gemcitabine-based therapy, which has not yet been given
- blocker: Ten prior irinotecan-containing cycles with recurrence at 4 months raise a topoisomerase-1 cross-resistance question the NAPOLI-1 population does not answer

Carried for the decision after this one: if a gemcitabine-based second line is used and fails, this becomes the on-label subsequent therapy. The caveat the label does not carry is that this tumor has already seen 10 irinotecan-containing cycles, and how much activity a liposomal formulation recovers after that has no direct answer in the NAPOLI-1 population, which was largely irinotecan-naive.

Key evidence.

- NAPOLI-1 (Wang-Gillam 2016) · randomized phase 3, open-label · n=417 · Median OS 6.1 vs 4.2 months for 5-FU/LV alone, HR 0.67 (95% CI 0.49-0.92), P=0.012

Toxicities that would change the decision. Grade 3-4 neutropenia 27%, diarrhea 13%, fatigue 14% in the combination arm

Alongside the targeted options. May be foreclosed by a targeted option. IBI343 carries an exatecan payload, another topoisomerase-1 poison. In either order, the second topoisomerase-1 exposure faces possible cross-resistance, and no published data settle that sequence.

Last verified. 2026-08-12

References. [fda:liposomal-irinotecan-post-gemcitabine-mPDAC-2015-10-22](#); [guideline:NCCN Pancreatic Adenocarcinoma v2.2026](#); [pmid:26615328](#)

Biliary decompression and GI symptom management, if obstruction develops

Priority: medium **Modality:** Supportive care **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- NCCN Principles of palliation and supportive care (partial population match · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Biliary or gastric outlet obstruction from pancreatic cancer: endoscopic decompression preferred where feasible, with percutaneous and surgical routes for selected cases.

Fit to this patient. Not determinable from the record The profile has no bilirubin, no cholangitis history, and no operative report, so neither the biliary anatomy after the December 2025 reconstruction nor any sign of current obstruction is on file. The missing facts are a baseline hepatic panel and the operative anatomy.

A recurrence in the pancreatic remnant can obstruct the biliary limb or the GI tract as it grows, and the standard response is endoscopic decompression rather than anything on the treatment tables. Nothing in the record says obstruction exists today, so this is carried for the moment it appears: the first bilirubin rise or gastric-outlet symptom. It is worth anticipating because post-surgical anatomy can make endoscopic access slower to arrange.

Toxicities that would change the decision. Procedure-related cholangitis and stent occlusion over time; altered post-surgical anatomy can force a percutaneous or EUS-guided route

Last verified. 2026-08-12

References. guideline:NCCN Pancreatic Adenocarcinoma v2.2026

Standard options behind an open gate

Endorsed for this patient's situation, but gated on a result that is missing, pending, or below threshold. The gate has to close before the option is real.

Chemoradiation or SBRT to an isolated operative-bed recurrence

Priority: high **Modality:** Radiotherapy **Line:** Not line-indexed **Intent:** disease-control intent

What makes it standard.

- NCCN Recurrence pathway option: chemoradiation or SBRT if not previously given, for local recurrence in the pancreatic operative bed (partial population match · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Local recurrence in the pancreatic operative bed without metastatic disease: systemic therapy with or without chemoradiation or SBRT (if radiotherapy was not previously given); clinical trial preferred.
- ASTRO Conditional recommendations (2019 clinical practice guideline) (different population · ASTRO 2019)
 - covers: Locally advanced pancreatic cancer: systemic chemotherapy followed by chemoradiation or SBRT in selected patients; written for intact primaries rather than operative-bed recurrence.

Fit to this patient. Conditionally eligible This patient has never been irradiated, so the pathway's 'if not previously given' condition is met. What is not yet established is the load-bearing fact: whether restaging reads this as an isolated local recurrence or as metastatic disease, and whether the vessels involved permit a meaningful dose.

- blocker: Restaging has not yet distinguished isolated local recurrence from metastatic disease
- blocker: Likely vascular invasion constrains dose, fractionation, and technique

If restaging reads this as an isolated operative-bed recurrence, radiotherapy is the standard local option, usually alongside or after systemic therapy, and this patient's lack of prior radiation keeps the pathway's condition met. The supporting evidence is institutional series rather than randomized data, and the likely vascular invasion constrains what dose is achievable. Re-resection of an isolated local recurrence is reported from high-volume centers but is not carried by NCCN or ESMO as a standard option, so it is not listed as one here.

Toxicities that would change the decision. GI toxicity of upper-abdominal radiation after a reconstruction, with duodenal or jejunal limb dose as the usual constraint

Alongside the targeted options. Competes for the same line of therapy. The staging call that makes this option live, locally recurrent disease without metastases, is the same call that closes the daraxonrasib expanded-access protocol, RASolute 303, and the ASP2138 monotherapy arms, while ASP5834 accepts locally advanced unresectable disease. Recent radiotherapy is itself an exclusion window in several protocols, so the timing question runs in both directions.

Last verified. 2026-08-12

References. guideline:NCCN Pancreatic Adenocarcinoma v2.2026; pmid:31474330

Olaparib maintenance (germline BRCA-gated)

Priority: medium **Modality:** Targeted therapy **Line:** Maintenance **Intent:** disease-control intent

What makes it standard.

- FDA On-label approval (2019) (partial population match · FDA approval 2019-12-27)
 - covers: Maintenance treatment of adults with deleterious or suspected deleterious germline BRCA-mutated metastatic pancreatic adenocarcinoma whose disease has not progressed on at least 16 weeks of a first-line platinum-based chemotherapy regimen; BRACAnalysis CDx is the companion diagnostic.
- ASCO Guideline recommendation (partial population match · ASCO Guideline Update 2020)
 - covers: Olaparib maintenance for patients with germline BRCA mutation and metastatic pancreatic cancer without progression after at least 16 weeks of first-line platinum-based chemotherapy.
- NCCN Maintenance option, germline BRCA1/2-mutated metastatic disease after platinum response (partial population match · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Germline BRCA1/2-mutated metastatic pancreatic adenocarcinoma without progression on first-line platinum-based therapy.

Fit to this patient. Conditionally eligible Both gates are open: germline status is unknown because the dedicated panel has never been drawn (incidental tumor-NGS reporting does not rule these genes out), and the label's platinum criterion is unmet as written, since the recurrence came about 4 months after platinum-containing adjuvant therapy rather than after 16 or more weeks of first-line platinum without progression.

- blocker: Germline BRCA1/2 status unknown; dedicated multigene panel never drawn
- blocker: Label requires no progression on ≥ 16 weeks of first-line platinum, which this course does not satisfy as written

- blocker: Recurrence 4 months after platinum-containing adjuvant therapy argues against the platinum sensitivity the strategy assumes

Biomarker gate. Germline BRCA1/2 pathogenic variant (PALB2 by extension, off-label) Deleterious or suspected deleterious germline BRCA1/2 variant by BRACAnalysis CDx or equivalent dedicated germline panel (never measured)

The one biomarker-gated FDA label in this disease, kept visible chiefly to keep the germline panel from slipping. Pre-test probability is roughly 5-7% for germline BRCA1/2 in unselected PDAC, and even in a carrier the demonstrated benefit is progression-free time (HR 0.53) with final OS flat at 19.0 vs 19.2 months. The label as written also presumes a platinum response this course has not shown, so any use here would be argued case by case after a positive panel.

Key evidence.

- POLO (Golan 2019) · randomized phase 3, double-blind, maintenance · n=154 · PFS 7.4 vs 3.8 months, HR 0.53 (95% CI 0.35-0.82), P=0.004
- POLO final OS (Kindler 2022) · final overall survival analysis · n=154 · Median OS 19.0 vs 19.2 months, HR 0.83, P=0.3487; no survival gain

Toxicities that would change the decision. Grade ≥ 3 AEs 40% vs 23% on placebo (anemia, fatigue, nausea), 5% discontinuation

Alongside the targeted options. Competes for the same line of therapy. Same strategy as rank 5 of the experimental ranking; it sits in both tables because it is FDA-approved and biomarker-gated. The blood draw that closes its gate is part of the workup bundle at the top of both tables.

Last verified. 2026-08-12

References. fda:olaparib-germline-BRCA-mutated-metastatic-pancreatic-adenocarcinoma-2019-12-27; pmid:31157963; pmid:35834777; pmid:32755482; guideline:NCCN Pancreatic Adenocarcinoma v2.2026

Already received

Carried so the report reads as a complete account of the standard options rather than a list that quietly omits what has been tried.

FOLFIRINOX / mFOLFIRINOX re-challenge

Priority: low **Modality:** Systemic chemotherapy **Line:** Second line **Intent:** life-prolonging intent

What makes it standard.

- NCCN Preferred regimen, first-line metastatic or locally advanced disease, good performance status (FOLFIRINOX or mFOLFIRINOX) (different population · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Treatment-naïve metastatic or locally advanced pancreatic adenocarcinoma with good performance status; the recurrence pathway routes recurrence within 6 months of adjuvant completion to an alternative regimen instead.

- ESMO I, A first line; ESMO-MCBS 5 (vs gemcitabine) (different population · ESMO CPG 2023; Express Update, April 2025)
 - covers: Treatment-naive metastatic pancreatic cancer, fit patients.

Fit to this patient. Already received Ten total cycles across the neoadjuvant and adjuvant settings, with recurrence about 4 months after completion.

Prior exposure. mFOLFIRINOX x6 neoadjuvant in 2025 with a partial response, then R0 resection in December 2025 (0/34 nodes, CAP tumor regression grade 2), then adjuvant FOLFIRINOX x4 completed per plan in March 2026. Treatment stopped at planned completion, not for toxicity or progression. A new pancreatic body mass appeared in July 2026, about 4 months later, which the NCCN recurrence pathway treats as progression through the regimen for sequencing purposes. Residual oxaliplatin neuropathy persists and has not been graded.

Carried so the record is explicit about why the strongest first-line regimen in this disease is not being re-offered: the patient has had 10 total cycles across the neoadjuvant and adjuvant settings, and recurrence 4 months after completion reads as functional resistance under the guideline's 6-month convention. Re-challenge in this window has no guideline carriage and would re-expose an already injured peripheral nervous system to oxaliplatin.

Key evidence.

- PRODIGE 4 / ACCORD 11 (Conroy 2011) · randomized phase 3, first-line metastatic · n=342 · Median OS 11.1 vs 6.8 months vs gemcitabine, HR 0.57 (95% CI 0.45-0.73), P<0.001
- PRODIGE 24 (Conroy 2018) · randomized phase 3, adjuvant · n=493 · Median DFS 21.6 vs 12.8 months vs gemcitabine, HR 0.58 (95% CI 0.46-0.73), P<0.001

Toxicities that would change the decision. Oxaliplatin re-exposure into an already injured peripheral nervous system

Last verified. 2026-08-12

References. guideline:NCCN Pancreatic Adenocarcinoma v2.2026; pmid:40287191; pmid:21561347; pmid:30575490

Assessed and set aside

Screened against this patient and found not to apply. Listed for the audit trail.

NALIRIFOX (liposomal irinotecan + oxaliplatin + 5-FU/leucovorin)

Priority: low **Modality:** Systemic chemotherapy **Line:** First line **Intent:** life-prolonging intent

What makes it standard.

- FDA On-label approval (2024) (partial population match · FDA approval 2024-02-13)
 - covers: First-line treatment of adult patients with metastatic pancreatic adenocarcinoma: liposomal irinotecan in combination with oxaliplatin, fluorouracil, and leucovorin.
- NCCN Preferred regimen, first-line metastatic disease, good performance status (different population · NCCN Pancreatic Adenocarcinoma v2.2026 (verified 22 April 2026; v3.2026 posted, not read))
 - covers: Treatment-naive metastatic pancreatic adenocarcinoma with good performance status; the same

guideline's recurrence pathway routes recurrence within 6 months of adjuvant completion to an alternative regimen.

- ESMO Preferred first-line option for fit patients (Express Update); ESMO-MCBS 2 (vs gemcitabine-nab-paclitaxel) (different population · ESMO Express Update, April 2025)
 - covers: Treatment-naive metastatic pancreatic cancer, fit patients.

Fit to this patient. Likely ineligible The regimen re-runs fluorouracil, oxaliplatin, and irinotecan within months of the tumor progressing through 10 cycles of the same three drug classes, and the ungraded oxaliplatin neuropathy independently gates a platinum re-exposure.

- blocker: Recurrence about 4 months after completing 10 cycles of 5-FU/oxaliplatin/irinotecan-based therapy; NCCN routes this to an alternative regimen
- blocker: Ungraded residual oxaliplatin neuropathy gates oxaliplatin re-exposure
- blocker: NAPOLI 3, the approval basis, enrolled treatment-naive metastatic patients

On paper the 2024 label covers first-line metastatic disease, and a patient entering her first metastatic line could be read into it. In practice this regimen re-runs fluorouracil, oxaliplatin, and irinotecan within months of the tumor progressing through 10 cycles of the same three classes, and the NCCN recurrence pathway routes a recurrence inside 6 months of adjuvant completion away from exactly that. Set aside for re-exposure and neuropathy reasons, not for lack of an approval.

Key evidence.

- NAPOLI 3 (Wainberg 2023) · randomized phase 3, open-label, first-line metastatic · n=770 · Median OS 11.1 vs 9.2 months vs gemcitabine/nab-paclitaxel, HR 0.83 (95% CI 0.70-0.99), P=0.036

Toxicities that would change the decision. Grade ≥ 3 TEAEs 87% in NAPOLI 3, including the oxaliplatin neurotoxicity this patient already carries a residue of

Last verified. 2026-08-12

References. [fda:liposomal-irinotecan-NALIRIFOX-first-line-mPDAC-2024-02-13](#); [guideline:NCCN Pancreatic Adenocarcinoma v2.2026](#); [pmid:40287191](#); [pmid:37708904](#)